



Chronic Pain



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Your Reports Are Normal. Then Why Does It Hurt This Much?

7 min read · Apr 17, 2026



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The Question That Doesn't Go Away

A few days ago, someone said something that has stayed with me in a way that many conversations don't.



They had been moving through the healthcare system for months, possibly longer consultations, tests, scans, second opinions. By the time they spoke, there was a

kind of exhaustion in their voice, not just from the pain itself but from the process of trying to explain it.

“I’ve seen five doctors. All my reports are normal. But I still can’t get through a full day without pain. So what exactly is wrong with me?”

It is a deceptively simple question. One that sits at the intersection of biology, perception, and systems that are often unequipped to hold that complexity. And perhaps more importantly, it is a question that many people living with chronic pain find themselves asking not once, but repeatedly, and often without receiving an answer that feels complete.

A Global Conversation, A Familiar Pattern

On 8 March 2026, through *Blooming in Pain*, we brought together a global group of participants people living with chronic pain, caregivers, clinicians, therapists, and advocates from more than 20 countries.

The intention was not to arrive at a set of solutions or to produce a consensus. Instead, it was to create a space where pain could be spoken about in ways that are often not possible within clinical settings where time is limited, language is constrained, and the expectation is to diagnose and treat rather than to sit with uncertainty.

What emerged from that conversation was not new in the sense of discovery. There were no surprising breakthroughs or radically different frameworks. What emerged instead was a pattern consistent, repeated, and difficult to ignore. Across geographies, healthcare systems, and personal histories, people were describing remarkably similar experiences. Not necessarily in terms of symptoms or diagnoses, but in the ways in which their pain was received, interpreted, and often dismissed.

When the Model of Pain Falls Apart

Part of the difficulty lies in how we have been taught to understand pain itself. There is a deeply ingrained expectation that pain functions as a direct and reliable signal of injury. Something goes wrong in the body, pain appears as a warning, the underlying issue is identified and treated, and the pain subsides as healing occurs.

This model works reasonably well for acute conditions. It is intuitive, linear, and aligns with how most healthcare systems are structured. But it begins to fall apart in the context of chronic or persistent pain, where the relationship between injury and experience is far less straightforward.

Pain is not simply a readout of tissue damage. It is an experience shaped by a range of factors that extend beyond the immediate physical state of the body stress, sleep, past experiences, emotional states, beliefs about pain, social environments, and the broader context of a person's life.

In this sense, pain is better understood not as a direct signal, but as a protective response one that can, over time, become amplified or sensitised. In chronic conditions, the nervous system may begin to behave like an overactive alarm, continuing to signal danger even in the absence of a clear or ongoing injury.

This does not make the pain any less real. If anything, it makes it more complex.

When “Nothing Is Wrong” Becomes a Response

However, complexity is not something our systems handle particularly well.

Healthcare systems, by design, tend to prioritise clarity and certainty. They are structured around identifying discrete problems and applying targeted interventions. This works effectively when the problem can be located, measured, and addressed within a defined framework.

But when tests return as “normal,” when scans do not provide clear answers, and when symptoms persist despite treatment, the system begins to reach its limits. And it is often at this point that the burden shifts subtly but significantly from the system to the individual.

Participants spoke about the kinds of responses they encountered in these moments: “there’s nothing wrong,” “your reports are fine,” or “it might just be stress.” In some cases, this extended to suggestions that the pain was “in the head.”

Even when not intended to dismiss, these responses create a rupture between experience and validation. Over time, this can lead individuals to question not only the system, but themselves.

What People Are Actually Looking For

One of the more striking insights from the conversation was that people were not only seeking treatment. In many cases, they had already pursued multiple forms of treatment, often without lasting relief.

What they were seeking was something both simpler and more difficult to provide.

They were seeking understanding.

They wanted to know why the pain persisted even when nothing “showed up.” They were looking for language that could help them explain their experience to doctors, to families, to employers, and sometimes to themselves. They were looking for validation that their pain was real, even in the absence of clear medical evidence. And they were looking for direction a way to move forward that did not depend entirely on eliminating the pain.

Without language, pain becomes isolating. And isolation often becomes part of the condition itself.

A Shift in the Question

Another theme that emerged, though more quietly, was a shift in how people begin to frame their relationship with pain over time.

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For many, the initial approach is focused on finding a cure identifying the cause and eliminating the pain. But when this does not happen, when the pain persists despite multiple attempts at treatment, the question begins to change.

It moves, sometimes reluctantly, from “How do I get rid of this?” to “How do I live with this?”

This is not an easy shift. Nor is it one that should be romanticised. But it reflects the reality many people are navigating.

Beyond a Medical Problem

What becomes increasingly clear through these conversations is that chronic pain cannot be understood or addressed as a purely medical issue. While it is often approached through the lens of diagnosis and treatment, the experience itself extends far beyond the boundaries of clinical care.

Pain shapes how people move through their daily lives. It affects their ability to work, sustain relationships, and participate in routines that once felt ordinary. Over time, it can also influence how individuals see themselves what they believe they are capable of, and what they begin to expect from the future.

For this reason, addressing chronic pain requires more than improving treatments or refining diagnostic tools. It calls for systems that are able to recognise complexity, that are comfortable with uncertainty, and that are willing to engage with the person as a whole rather than reducing the experience to a set of symptoms.

This is not an easy shift, particularly in systems that are already overburdened. But without it, a large part of the experience of chronic pain remains unaddressed.

Starting With Belief

In this context, one of the most meaningful changes may not be technical or procedural, but relational.

It lies in how people are first received when they speak about their pain.

For many participants, the turning point in their journey was not a specific treatment, but an interaction in which they felt genuinely heard. A clinician, therapist, or even a peer who did not immediately attempt to explain away the pain, but instead acknowledged it without hesitation.

To say “I believe you” may appear simple, but it carries significant weight in situations where individuals have repeatedly encountered doubt or dismissal. It helps restore a sense of legitimacy to an experience that has often been questioned, and creates a foundation from which further exploration medical or otherwise can take place.

Similarly, recognising that pain can be real even when it is not visible, and that the absence of clear answers does not imply the absence of a problem, allows for a more honest and constructive engagement with the condition.

These shifts do not resolve the pain in themselves. However, they can fundamentally change how care is experienced, and whether individuals feel supported in navigating what is often a long and uncertain process.

Rethinking the Question

Perhaps the most significant reframing that emerged from the conversation was not a solution, but a different way of approaching the experience of pain.

Many people begin by asking, understandably, “What is wrong with me?” It is a question shaped by the expectation that there must be a clear cause something identifiable that can be named and addressed.

However, when that clarity does not emerge, the question can become a source of frustration rather than understanding.

An alternative that some participants arrived at, gradually and often with difficulty, was to ask a different question: “Given everything I am experiencing, why might this pain make sense?”

This shift does not eliminate uncertainty, nor does it reduce the intensity of pain. What it does is create space for a more nuanced understanding one that takes into account the multiple factors that shape pain, rather than searching for a single, definitive explanation.

In doing so, it allows individuals to move away from a cycle of self-doubt and towards a framework that is more grounded in their lived reality.

What We Choose to Acknowledge

Chronic pain continues to sit in an uncomfortable space within our systems. It does not always present clear evidence, it does not follow predictable timelines, and it often resists the kind of categorisation that healthcare structures rely on. As a result, the responsibility of making sense of it is frequently pushed back onto those who are already living with it.

What this conversation made evident, however, is that the challenge is not only about improving treatment. It is about expanding our capacity to understand

experiences that do not fit neatly into existing frameworks. It is about recognising that the absence of visible proof does not diminish the reality of pain, and that uncertainty should not automatically lead to dismissal.

If there is a shift to be made, it lies as much in how we respond as in what we know. Listening more carefully, speaking more precisely, and creating space for complexity are not peripheral to care they are central to it.

Because for many people, the most difficult part is not only the persistence of pain, but the experience of having to continually justify it.

And that is something systems can change.

*This article draws from a global webinar hosted under Blooming in Pain, with contributions from **Dr. Deepak Ravindran**, **Professor Cormac Ryan**, **Vachasamrita**, and **Dr. Shilpi**. The conversation was part of a broader effort by Blooming in Pain to build language, community, and understanding around chronic pain.*

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Constantly being in acute pain can be daunting and often a lonely journey. It is a space to rise and fall in stories of pain, healing, and co-existence.

